

pharmaceutical companies. We did not take methadone into consideration, since it is used for drug-users' detoxification.

By contrast with what was expected, opioid consumption did not increase after the approval of the new law. Sales of only one preparation, more introduced into the market after the law's approval, rose sharply to increase in its selling. Morphine consumption, as a sustained release formulation, substantially decreased. Therefore, freedom of prescribing should not be seen as automatically leading to increases in opioid consumption. This has been probably considered an alibi for many years. Lack of knowledge, ignorance, patient-related barriers, fears about addiction, and failure to educate doctors and other health-care workers by national governments in cancer-pain management remain the major reasons for unsatisfactory treatment. The difficulty of under-education of physicians about pain assessment and management should be solved by the integration of relevant postgraduate training programmes.

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Continuing influence of tobacco industry in Germany

Sir—Mark Neuman and colleagues (April 13, p 1323)¹ highlight how industry lobbying tactics have undermined European tobacco control, with certain European Union (EU) member states, particularly Germany, playing key parts.

They focus on Germany's government under Chancellor Kohl (1982–98), but the long-standing industry influence seems to be continuing under the present government. This influence has relevance far beyond Germany, since it undermines European tobacco-control policies and hinders attempts to strengthen global tobacco control

via WHO's Framework Convention on Tobacco Control (FCTC).

Germany's resistance to effective tobacco control has been attributed to a reaction to the Nazi's strong opposition to smoking. However, despite libertarian views being more strongly held and advocacy weaker in Germany than in the rest of northern Europe, Neuman and colleagues show that the close relation between the cigarette industry and the Kohl government also played a fundamental part.

Unfortunately, industry links with government seem to have continued since Kohl left office.² Tobacco-industry journals still describe Germany as a strong supporter of the tobacco industry.³ Schröder's government has continued to support the advertising-ban challenge launched by Kohl before leaving office, abstained from the European Council vote on the Tobacco Regulations Directive (along with only Luxembourg), and now plans to challenge this Directive in the European Court of Justice.

The government has attracted further ridicule by accepting €11.8 million of industry funding for a 5-year tobacco-control programme⁴ that purports to prevent children and adolescents from smoking. The industry's documents reveal its true attitude and the importance it attaches to encouraging rather than preventing youth smoking.⁵ Moreover, the industry contract with the German government explicitly stipulates that the cigarette industry, their products, or cigarette trading must not be discriminated against, and adult smokers must not be denigrated, precluding the campaign from preventing youth smoking.

Germany presents its opposition to EU tobacco policies in terms of subsidiarity, claiming that such measures should be dealt with at member-state level. At the Warsaw Convention in February, 2002, David Byrne, EU Health and Consumer Safety Commissioner, argued that this position was no longer tenable; he stated, in relation to the FCTC, "here . . . they finally have the opportunity to practice what they preach. And how do they react? Well, surprisingly they continue to block proposals on issues such as a complete ban on advertising."

Neuman and colleagues suggest the real reason for the German government's resistance lies with its industry links. It is unfortunate that the German government places the tobacco industry's demands ahead of

the health of its population, but that is a matter for the German public. The government should not, however, be allowed to block action that would benefit people's health elsewhere. Germany is effectively neutralising the EU's position on the FCTC, and although some member states have been content to shelter behind its position, others grow frustrated. To advance European tobacco control and adopt a positive stance on the FCTC, Germany's EU partners must make it clear that such behaviour is no longer acceptable if the rising death toll from tobacco is to be curtailed.

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Reply from the Drug Commissioner of the German Federal Government

Sir—Anna Gilmore and colleagues misrepresent the German Federal Government's contract with the cigarette industry and make exaggerated political insinuations and assumptions. I protest on behalf of the German Federal Government against these accusations.

Germany supports the health policy objectives to reduce tobacco consumption by statutory and preventive measures. It declared this to be one of the five main health objectives. A national action plan on tobacco will have an overall aim of making non-smoking the norm, of using statutory measures to render access more difficult, and to create structures for tobacco control. A national prevention programme will ensure that preventive measures are implemented consistently.

It is in this political context that the agreement between the Federal Ministry for Health and the cigarette industry must be situated. The industry constitutes a financial building block within the framework of a national prevention fund to which, alongside the

statutory health insurance funds and affected organisations, it is expected to make a contribution. The contract is open for public perusal on the internet.¹ Consequently, the terms of the agreement are completely transparent.

The Federal Government has had experiences with health policy through voluntary agreements and legal restrictions. It maintains a critical dialogue with the tobacco industry about the assumption of responsibility for the health damage caused by tobacco products. The inclusion of financial resources from industry in measures and programmes introduced by policy makers is, in principle, always to be welcomed. This approach is clearly recommended by the European Commission on the Prevention and Targeted Control of Tobacco Consumption for implementation of new directives at national level.

However, essentially, a change is required in society's attitude towards non-smoking, and can be brought about only by the cooperation of the groups, institutions, and actors involved. Packages must be put together by individual EU nations, taking into account their own historical, cultural, and social backgrounds. This approach is not a political contradiction, but a desired result.

Germany seeks also to introduce additional more stringent regulations on smoking. The Protection of Young People in Public Act now includes a ban on the distribution of tobacco products to people younger than age 16 years. Also cigarette vending machines must be secured to prevent access for this age group. Advertisements for tobacco and alcoholic beverages cannot be shown in cinemas before 1800 h. Under the new Work Place Ordinance, employees will be granted a legal right to have a smoke-free workplace.

Germany advocated within the EU the elimination of subsidies for tobacco cultivation, but was, regrettably, unsuccessful. The Government also supports several national and international prevention projects in schools, municipalities, and medical practices to introduce extensive and targeted group-specific programmes, along public-health lines, for smoking prevention.

In view of these measures, it is unjustified to speak of an irresponsible health-policy attitude at the expense of the population's health or the continuation of the tobacco policy of the previous Federal Government. To clarify this stance, several talks have been held between EU Health

Commissioner, David Byrne, and Germany's federal minister for health.

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1 Bundesministerium für Gesundheit.
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Oct 7, 2002)

Visibility of research: FUTON bias

Sir—The availability of full-text articles on the internet has greatly improved ease of access to medical information. This development must be seen as a great benefit, but it may have generated a new type of bias.

Everyday information-seeking activities, especially by junior staff and students, often concentrate on research published in journals that are available as full text on the internet, and ignore relevant studies that are not available in full text, thus introducing an element of bias into their search result.

This bias I propose to call FUTON (Full Text On the Net) bias. It will not affect researchers who are used to comprehensive searches of published medical studies, but it does affect staff and students with limited experience in doing searches. This bias may have the same effect in daily clinical practice as publication bias¹ or language bias² when doing systematic reviews of published studies.

I also believe that the FUTON bias will affect the standing of medical journals, since full-text journals on the internet are more visible and may be cited more frequently in the future than the traditional print journals. Publishers of medical journals should feel encouraged to make the content of their journals available as full text to avoid losing out to their competitors.

	Pre-Medline + Medline	EMBASE
Words searched in report title		
Clinical governance		
Items found	202	224
Items without abstract	130 (64%)	127 (57%)
Torture		
Items found	558	224
Items without abstract	415 (74%)	127 (56%)
Myocardial infarction		
Items found	8615	8316
Items without abstract	1787 (20%)	1526 (18%)
Number of items found for new and more established topics		

This trend will gain further strength if the journals that make preprints available on the internet or publish internet-only versions of reports are taken into account.

I also draw attention to the no abstract available (NAA) bias, which makes publications that have no abstract (eg, letters, editorials) on Medline or other electronic databases less attractive to peruse (and consequently less cited) than those with abstracts. I propose that abstracts be included in all substantial publications, such as editorials and research letters.

The NAA bias affects publications on new, peripheral, and under-discussion subjects more than established topics covered in substantive reports. A comparison of the subject clinical governance and torture, with myocardial infarction, for example, substantiates the effect of the NAA bias (table).

Visibility of research is also reduced because some important medical journals are not indexed on some major databases. The *International Journal of Obstetric Anaesthesia*, for example, is not indexed on Medline, although it is ranked eighth among anaesthetic journals (impact factor 2001, 1.187). Its 600 or more reports, including some 80 randomised controlled trials, to date cannot be seen by researchers, if they restrict their search to Medline.

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DEPARTMENT OF ERROR

Intermittent administration of iron and sulfadoxine-pyrimethamine to control anaemia in Kenyan children: a randomised controlled trial—In this Article by H Verhoef and colleagues (published online August 20, 2002. <http://image.thelancet.com/extras/01art9014web.pdf>), the fourth sentence of the introduction should have read: "Oral iron supplementation (2 mg/kg) given daily to infants aged 8–24 weeks in an area of high, perennial transmission did not increase the incidence of malaria".

Chronic cough and ear wax—In this case report by F Jegoux and colleagues (Aug 24, p 618), the first sentence should have read "A 7-year-old boy had a right-sided tympanoplasty for attical cholesteatoma in 1988".